



DPR: Adolescent Wellness

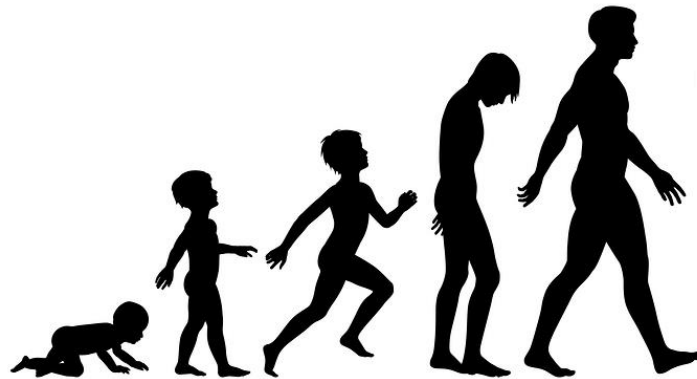
Principles and Practices of Osteopathic
Medicine I

Thomas Chan D.O., FAAP
Associate Professor
Dept of Clinical Specialties
tchan02@nyit.edu

Session Objectives

- Identify how the stages of adolescent development contribute to certain behaviors and risk
- Understand important aspects of gathering a history from an adolescent with regards to confidentiality and the limits of confidentiality.
- Understand building rapport using a HEADS assessment, and risk assessment for problems common in the adolescent population
- Describe and understand adolescent pubertal physical development
- Understand the elements of an adolescent physical exam based on common conditions encountered in adolescence

- Adolescents
 - Transition from childhood to adulthood
 - Ages 11 to 21
 - The age at onset of puberty varies but the stages always go in the same sequence



Adolescents

- There is a struggle for identity and independence which can lead to high risk behavior



Understanding Medical Care for the Adolescent

- Traditional Care
- Transitional Care
- Treasure hunter
- Troubling problems
- Take The Time



10 Leading Causes of Death by Age Group, United States – 2014

Rank	Age Groups										Total
	<1	1-4	5-9	10-14	15-24	25-34	35-44	45-54	55-64	65+	
1	Congenital Anomalies 4,746	Unintentional Injury 1,216	Unintentional Injury 730	Unintentional Injury 750	Unintentional Injury 11,836	Unintentional Injury 17,357	Unintentional Injury 16,048	Malignant Neoplasms 44,834	Malignant Neoplasms 115,282	Heart Disease 489,722	Heart Disease 614,348
2	Short Gestation 4,173	Congenital Anomalies 399	Malignant Neoplasms 436	Suicide 425	Suicide 5,079	Suicide 6,569	Malignant Neoplasms 11,267	Heart Disease 34,791	Heart Disease 74,473	Malignant Neoplasms 413,885	Malignant Neoplasms 591,699
3	Maternal Pregnancy Comp. 1,574	Homicide 364	Congenital Anomalies 192	Malignant Neoplasms 416	Homicide 4,144	Homicide 4,159	Heart Disease 10,368	Unintentional Injury 20,610	Unintentional Injury 18,030	Chronic Low. Respiratory Disease 124,693	Chronic Low. Respiratory Disease 147,101
4	SIDS 1,545	Malignant Neoplasms 321	Homicide 123	Congenital Anomalies 156	Malignant Neoplasms 1,569	Malignant Neoplasms 3,624	Suicide 6,706	Suicide 8,767	Chronic Low. Respiratory Disease 16,492	Cerebro-vascular 113,308	Unintentional Injury 136,053
5	Unintentional Injury 1,161	Heart Disease 149	Heart Disease 69	Homicide 156	Heart Disease 953	Heart Disease 3,341	Homicide 2,588	Liver Disease 8,627	Diabetes Mellitus 13,342	Alzheimer's Disease 92,604	Cerebro-vascular 133,103
6	Placenta Cord. Membranes 965	Influenza & Pneumonia 109	Chronic Low. Respiratory Disease 68	Heart Disease 122	Congenital Anomalies 377	Liver Disease 725	Liver Disease 2,582	Diabetes Mellitus 6,062	Liver Disease 12,792	Diabetes Mellitus 54,161	Alzheimer's Disease 93,541
7	Bacterial Sepsis 544	Chronic Low Respiratory Disease 53	Influenza & Pneumonia 57	Chronic Low Respiratory Disease 71	Influenza & Pneumonia 199	Diabetes Mellitus 709	Diabetes Mellitus 1,999	Cerebro-vascular 5,349	Cerebro-vascular 11,727	Unintentional Injury 48,295	Diabetes Mellitus 76,488
8	Respiratory Distress 460	Septicemia 53	Cerebro-vascular 45	Cerebro-vascular 43	Diabetes Mellitus 181	HIV 583	Cerebro-vascular 1,745	Chronic Low. Respiratory Disease 4,402	Suicide 7,527	Influenza & Pneumonia 44,836	Influenza & Pneumonia 55,227
9	Circulatory System Disease 444	Benign Neoplasms 38	Benign Neoplasms 36	Influenza & Pneumonia 41	Chronic Low Respiratory Disease 178	Cerebro-vascular 579	HIV 1,174	Influenza & Pneumonia 2,731	Septicemia 5,709	Nephritis 39,957	Nephritis 48,146
10	Neonatal Hemorrhage 441	Perinatal Period 38	Septicemia 33	Benign Neoplasms 38	Cerebro-vascular 177	Influenza & Pneumonia 549	Influenza & Pneumonia 1,125	Septicemia 2,514	Influenza & Pneumonia 5,390	Septicemia 29,124	Suicide 42,773

Data Source: National Vital Statistics System, National Center for Health Statistics, CDC.
Produced by: National Center for Injury Prevention and Control, CDC using WISQARS™.



Centers for Disease
Control and Prevention
National Center for Injury
Prevention and Control

CDC

NYIT

College of
Osteopathic
Medicine

10 Leading causes of mortality- 2021 CDC

	<1	1-4	5-9	10-14	15-24	25-34	35-44	45-54	55-64	65+	All Ages
1	Congenital Anomalies 3,963	Unintentional Injury 1,299	Unintentional Injury 827	Unintentional Injury 915	Unintentional Injury 15,792	Unintentional Injury 34,452	Unintentional Injury 36,444	Covid-19 36,881	Malignant Neoplasms 108,023	Heart Disease 553,214	Heart Disease 695,547
2	Short Gestation 2,946	Congenital Anomalies 412	Malignant Neoplasms 347	Suicide 598	Homicide 6,635	Suicide 8,862	Covid-19 16,006	Heart Disease 34,535	Heart Disease 89,342	Malignant Neoplasms 446,354	Malignant Neoplasms 605,213
3	Sids 1,459	Homicide 309	Homicide 188	Malignant Neoplasms 449	Suicide 6,528	Homicide 7,571	Heart Disease 12,754	Malignant Neoplasms 33,567	Covid-19 73,725	Covid-19 282,457	Covid-19 416,893
4	Unintentional Injury 1,306	Malignant Neoplasms 282	Congenital Anomalies 171	Homicide 298	Covid-19 1,401	Covid-19 6,133	Malignant Neoplasms 11,194	Unintentional Injury 31,407	Unintentional Injury 33,471	Cerebrovascular 139,257	Unintentional Injury 224,935
5	Maternal Pregnancy Comp. 1,113	Heart Disease 116	Heart Disease 66	Congenital Anomalies 179	Malignant Neoplasms 1,323	Heart Disease 4,155	Suicide 7,862	Liver Disease 10,501	Diabetes Mellitus 18,603	Chronic Low. Respiratory Disease 120,152	Cerebrovascular 162,890
6	Placenta Cord Membranes 672	Perinatal Period 68	Covid-19 63	Heart Disease 132	Heart Disease 944	Malignant Neoplasms 3,615	Liver Disease 5,833	Diabetes Mellitus 7,597	Liver Disease 17,664	Alzheimer's Disease 117,922	Chronic Low. Respiratory Disease 142,342
7	Bacterial Sepsis 557	Cerebrovascular 55	Chronic Low. Respiratory Disease 54	Covid-19 79	Congenital Anomalies 419	Liver Disease 1,833	Homicide 4,863	Suicide 7,401	Chronic Low. Respiratory Disease 17,620	Diabetes Mellitus 72,451	Alzheimer's Disease 119,399
8	Respiratory Distress 414	Covid-19 54	Cerebrovascular 35	Cerebrovascular 53	Diabetes Mellitus 345	Diabetes Mellitus 1,285	Diabetes Mellitus 2,961	Cerebrovascular 5,755	Cerebrovascular 14,634	Unintentional Injury 69,003	Diabetes Mellitus 103,294
9	Circulatory System Disease 402	Influenza & Pneumonia 47	Septicemia 28	Chronic Low. Respiratory Disease 45	Complicated Pregnancy 214	Complicated Pregnancy 797	Cerebrovascular 2,189	Chronic Low. Respiratory Disease 3,174	Suicide 7,267	Nephritis 44,013	Liver Disease 56,585
10	Intrauterine Hypoxia 358	Benign Neoplasms 37	Influenza & Pneumonia 27	Diabetes Mellitus 39	Cerebrovascular 190	Cerebrovascular 624	Septicemia 1,108	Homicide 2,768	Septicemia 6,477	Parkinson's Disease 37,568	Nephritis 54,358

Applied Filters

Intent of Injury: All Deaths with drilldown to ICD codes

Years: 2021

Age: 1-14 in 5-year groups; 15-65+ in 10-year groups

Race: All Races

Number of Causes: 10

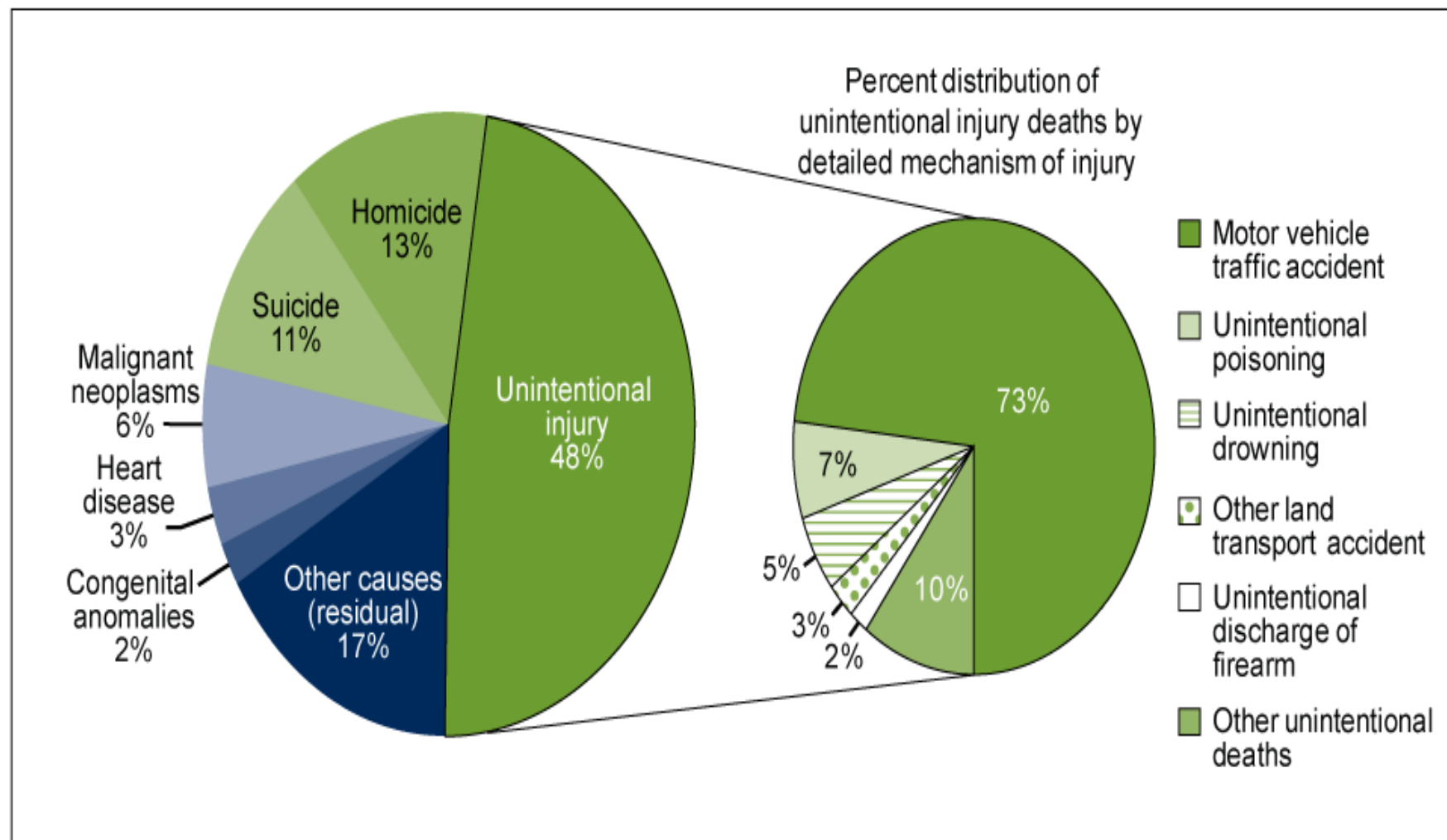
Race Year: 2001 - 2021 with No Race

Geography: United States

Sex: Both Sexes

Ethnicity: All Ethnicities

Figure 2. Percent distribution of all deaths to teenagers 12–19 years, by cause of death: United States, 1999–2006



SOURCE: National Vital Statistics System, Mortality.

Understand the Adolescent- Stages

Early (11-14yrs)

- Puberty starting, starting struggle for independence but still childish in many ways, same sex peer groups, starting interest in sexuality, concrete thinking (things black and white/ right and wrong)

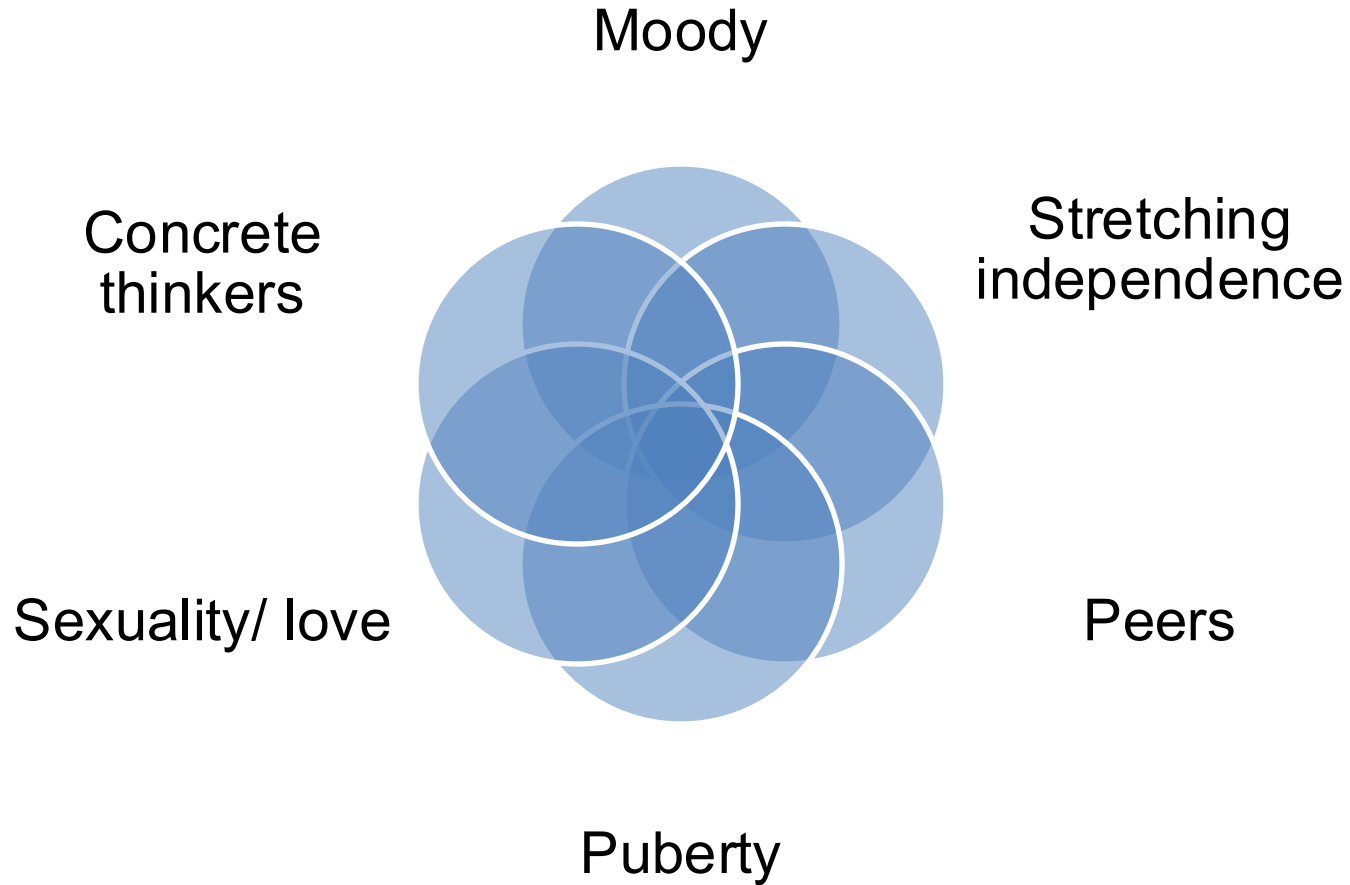
Middle (15-17yrs)

- Continued physical changes, independence progressing, love/passions, family conflict highest, mixed peer groups, maturing thought process

Late (18-21yrs)

- more mature self identity, thinking and independence, peers and family dynamics change, mature relationships, can think more about future and others, understand life may be grey, not as simple as black and white/ right and wrong

Adolescents are distinct



Build Rapport with Adolescent AND Parent

- Introductions
- Ask teen and parents if any concerns
 - Chit/ Chat- no confidential Questions yet
 - +/- Physical exam
 - Get a feel for the Temp in the room
 - Get a sense of cognitive and pubertal development
- At some point ask parent to leave the room. "I would like to talk to XXX alone for a bit, would you mind stepping out of the room"
- The younger the patient the more mindful you need to be esp 11 and 12 y/o



Adolescent Confidentiality

- The adolescent should be offered an opportunity for examination and counseling separate from parents/guardians, and the physician should encourage and assist the adolescent to involve parents or guardians in healthcare decisions.
- Physicians should deliver confidential health services in situations involving sexuality (including sexually transmitted infections, contraception, and pregnancy), substance use/abuse, and mental health to consenting adolescents.
- Adolescent patients should be made aware that certain situations and circumstances create limitations on guaranteed confidentiality. For example, detailing billing statements and Explanation of Benefits notices may be furnished to a guarantor/parent from a third party.
- **MANDATORY REPORTING:** Someone is in imminent danger, the suspicion or evidence of abuse, and the diagnosis of certain communicable diseases all MUST be reported to the proper authorities.
- If communication between the adolescent and parent cannot be facilitated, every effort should be made by physicians and their staff to ensure confidentiality within the limits of legal and ethical standards.

Confidentiality Statement

- When alone, ‘whatever is discussed will be kept confidential, I don’t tell your parents anything you don’t want me to. The only exception is if there is a danger to yourself or someone else.’

Confidentiality Limits

- Never make confidentiality unlimited
- Your patient needs to know that they can tell you things in confidence but they have to know that there are exceptions
 - Threat to others
 - Threat to self

STD's and Pregnancy stays confidential

H.E.A.D.S Assessment- “Interview ALONE”

Confidentiality statement

Home

Education, **E**mployment

Activities

Drugs, **D**rinking, **D**iet, **D**epression

Sex, **S**moking, **S**uicide

- Who lives at home? parents, siblings, relatives, friends
- Chores? Home responsibilities?
- Living situation, any Changes?
 - Divorce/ Separations/ new parental partners?
 - If parents not together, what are the arrangements?
- Feel safe at home?

Education/ Employment

- What grade? regular class/special-ed/honors
- Grades- get specific
 - Left back/ summer school
- Activities at school/ interests
- Friends?, changes in friends?
- Bullying?
- Safe at school?

Employment

- Working? How many hours/ schedule

Activities

- What do you like to do in your free time?
- What do you enjoy?
- Sports?
- Hobbies? Reading? Journaling? Writing?
- Computers? Gaming? Social media?

Ask about Sleep- how much, schedule of sleep. (Adolescents chronically sleep deprived)

-discuss sleep hygiene, esp Cellphones

History Gathering- Warning Signs

- Worsening school performance
- Chronic absence/ lateness
- Significant weight change
- Significant attitude/ personality change
- Social isolation
- Loss of joy in previous activities
- Change in friends/ Troubling Peers
- Change in patterns
(eat.sleep.appearance.activities)
- Incidents (suspensions, fights, accidents)
- Temper outbursts

Now for the nitty gritty....

- Deep breath...be cool....Let's ROCK!



- Ask and get specifics
 - How much, how often, situation? passed out?
 - Marijuana? Any Pills? Ecstasy/ Molly? Etc
 - Need to use to feel normal?
 - How do you get the money?
 - CRAFFT Screening
 - CAGE Questionnaire for alcohol

CAGE Alcohol Questionnaire

Share

CAGE is a 4-item questionnaire that can indicate potential problems with alcohol use.

Questions

Points

C : Have you ever felt you should Cut down on your drinking?

☐ Yes +1

☐ No +0

A : Have people Annoyed you to by criticizing your drinking?

☐ Yes +1

☐ No +0

G : Have you ever felt Guilty about your drinking?

☐ Yes +1

☐ No +0

E : Have you ever had a drink first thing in the morning (Eye opener)?

☐ Yes +1

CAGE score is obtained by adding the points (total points)

0 points

The CRAFFT Screening Interview

Begin: "I'm going to ask you a few questions that I ask all my patients. Please be honest. I will keep your answers confidential."

Part A

During the PAST 12 MONTHS, did you:

No Yes

- | | | |
|---|--------------------------|--------------------------|
| 1. Drink any <u>a</u> lcohol (more than a few sips)?
(Do not count sips of alcohol taken during family or religious events.) | ☐ | ☐ |
| 2. Smoke any <u>m</u> arijuana or hashish? | ☐ | ☐ |
| 3. Use <u>a</u> nything <u>e</u> lse to <u>g</u> et high?
("anything else" includes illegal drugs, over the counter and prescription drugs, and things that you sniff or "huff") | ☐ | ☐ |

For clinic use only: Did the patient answer "yes" to any questions in Part A?

No ☐

Yes ☐

Ask CAR question only, then stop

Ask all 6 CRAFFT questions

Part B

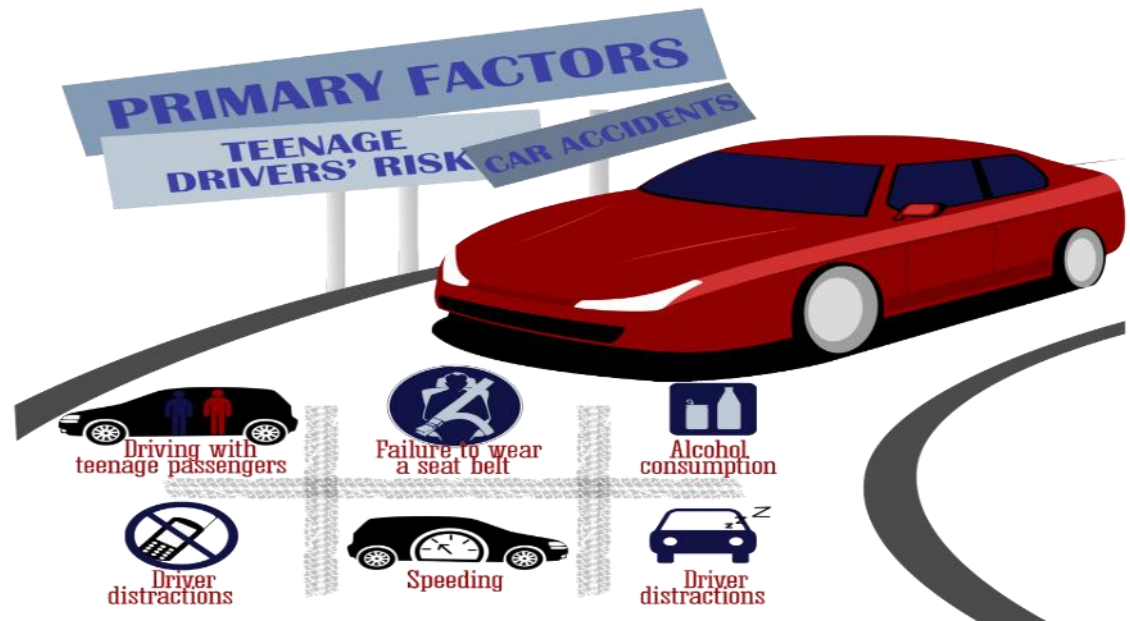
No Yes

- | | | |
|--|--------------------------|--------------------------|
| 1. Have you ever ridden in a <u>C</u> AR driven by someone (including yourself) who was "high" or had been using alcohol or drugs? | ☐ | ☐ |
| 2. Do you ever use alcohol or drugs to <u>R</u> ELAX, feel better about yourself, or fit in? | ☐ | ☐ |
| 3. Do you ever use alcohol or drugs while you are by yourself, or <u>A</u> LONE? | ☐ | ☐ |
| 4. Do you ever <u>F</u> ORGET things you did while using alcohol or drugs? | ☐ | ☐ |
| 5. Do your <u>F</u> AMILY or <u>F</u> RIENDS ever tell you that you should cut down on your drinking or drug use? | ☐ | ☐ |
| 6. Have you ever gotten into <u>T</u> ROUBLE while you were using alcohol or drugs? | ☐ | ☐ |

- **MVA #1 Killer of Teenagers**

- In 2014, 2,270 teens in the United States ages 16–19 were killed and 221,313 were treated in emergency departments for injuries suffered in motor vehicle crashes.¹ That means that six teens ages 16–19 died every day from motor vehicle injuries

- **Driving while intoxicated, impaired, distracted**



Underdiagnosed, not always obvious

- PHQ2 (start at 11 y/o and beyond)
 - if positive then follow with PHQ-9

PHQ-2 screening for Depression

Patient Health Questionnaire-2 (PHQ-2)

 Share

The PHQ-2 inquires about the frequency of depressed mood and anhedonia over the past two weeks. The PHQ-2 includes the first two items of the PHQ-9.

- The purpose of the PHQ-2 is to screen for depression in a “first-step” approach.
- Patients who screen positive should be further evaluated with the PHQ-9 to determine whether they meet criteria for a depressive disorder.

Over the **last 2 weeks**, how often have you been bothered by the following problems?

Not at all

Several days

More than half the days

Nearly every day

1. Little interest or pleasure in doing things

☐

0

☐

+1

☐

+2

☐

+3

2. Feeling down, depressed or hopeless

☐

0

☐

+1

☐

+2

☐

+3

PHQ-2 score obtained by adding score for each question (total points)

Interpretation:

- A PHQ-2 score ranges from 0-6. The authors identified a score of 3 as the optimal cutpoint when using the PHQ-2 to screen for depression.
- If the score is 3 or greater, major depressive disorder is likely.
- Patients who screen positive should be further evaluated with the [PHQ-9](#), other diagnostic instruments, or direct interview to determine whether they meet criteria for a depressive disorder.

PHQ-9 Screening tool

The Patient Health Questionnaire (PHQ-9)

Patient Name _____ Date of Visit _____

Over the past 2 weeks, how often have you been bothered by any of the following problems?

	Not At all	Several Days	More Than Half the Days	Nearly Every Day
1. Little interest or pleasure in doing things	0	1	2	3
2. Feeling down, depressed or hopeless	0	1	2	3
3. Trouble falling asleep, staying asleep, or sleeping too much	0	1	2	3
4. Feeling tired or having little energy	0	1	2	3
5. Poor appetite or overeating	0	1	2	3
6. Feeling bad about yourself - or that you're a failure or have let yourself or your family down	0	1	2	3
7. Trouble concentrating on things, such as reading the newspaper or watching television	0	1	2	3
8. Moving or speaking so slowly that other people could have noticed. Or, the opposite - being so fidgety or restless that you have been moving around a lot more than usual	0	1	2	3
9. Thoughts that you would be better off dead or of hurting yourself in some way	0	1	2	3

Source: SAMHSA.gov

Adolescent Depression- SIG E CAPS

- **Sleep disturbance**
- **Interests**
- **Guilt/ Self esteem**
- **Energy**
- **Concentration**
- **Appetite**
- **Psychomotor agitation/ retardation**
- **Suicidal ideation**

Suicide

Recognize increasing likelihood of action

- Comorbid conditions (mental illness, depression)
- Gender identity/other factors (bullying)
- Incarceration
- Self destructive thoughts
- Prior attempts
- Substance abuse (ETOH, Drugs)
- Giving away possessions
- Access to lethal means
- Hallucinations (commands)
- Signif changes/ stressors/ inciting events
- Recent death or suicide of friend/ relative

Assess Suicidal Ideation/ Homicidal ideation



Suicidal Ideation- assess severity, plan, previous attempts

- Educate, discuss Action plan if thoughts worsen, persists (988)

Homicidal Ideation- anger, plan, obsession

Imminent Danger to self or others: can break confidentiality

Try to have adolescent agree to discuss with parent so don't have to break Confidence if SI is high risk

Anxiety

- GAD 2
- GAD 7

Generalized Anxiety Disorder 7-item (GAD-7)

[Share](#)

The Generalized Anxiety Disorder 7-item (GAD-7) is a easy to perform initial screening tool for generalized anxiety disorder¹.

Over the **last 2 weeks**, how often have you been bothered by the following problems?

Not at all

Several days

More than half the days

Nearly every day

1. Feeling nervous, anxious or on edge

☐

0

☐

+1

☐

+2

☐

+3

2. Not being able to stop or control worrying

☐

0

☐

+1

☐

+2

☐

+3

3. Worrying too much about different things

☐

0

☐

+1

☐

+2

☐

+3

4. Trouble relaxing

☐

0

☐

+1

☐

+2

☐

+3

5. Being so restless that it is hard to sit still

☐

0

☐

+1

☐

+2

☐

+3

6. Becoming easily annoyed or irritable

☐

0

☐

+1

☐

+2

☐

+3

7. Feeling afraid as if something awful might happen

☐

0

☐

+1

☐

+2

☐

+3

Smoking

- Tobacco products- smoking, chewing
- Ask specifically about Vaping
- Get Details if yes
- If no, still tell them the dangers

- Gender identity/
preference
- Sexual Activity
 - Number of partners
 - Protection from
pregnancy/ STD's
 - Prior STD's



More questions...dig deeper if indicated

- Abuse- physical, sexual
- Intentional injury- cutting, burning
- Eating Disorders
- Inappropriate 'adult' encounters

Physical Exam

- Vitals
- Gen
- Skin
- Head, Eyes ,Ears, and Mouth
- Neck
- Heart
- Lungs
- Abdomen
- genitourinary system
- Musculoskeletal
- Neurologic system

General Assessment

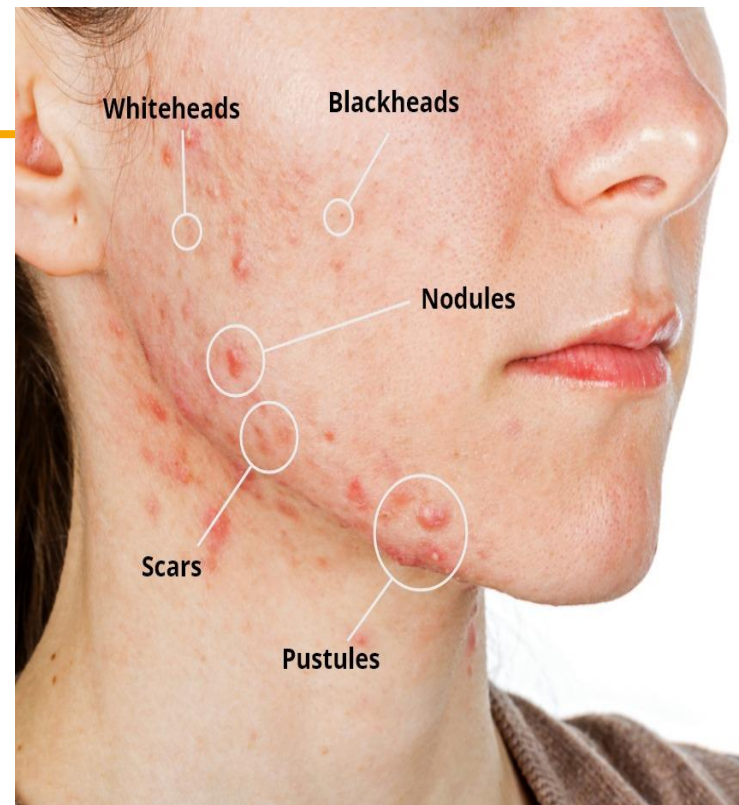
- Affect
- Attention, behavior
- Mood
- Hygiene, grooming
- Clothes
- Maturity

Document fairly and carefully

Skin

Examine the skin

- Acne,
- acanthosis nigricans
- Moles/ Benign nevi
(*discuss sun protection*)
- self-injury (cutting) or
signs of abuse
- Pallor/ anemia
- Tattoos
- Piercings (infected)
- Bruises



HEENT, NECK

- Visual acuity testing annually- done with SNELLEN chart
- Hearing testing
- Hair: lice, texture, hair loss
- Eyes: red, pale
- Ears: cerumen impaction
- Nose: epistaxis, turbinates, polyps
- Mouth: tonsils, pallor, dentition
- Neck: Lymph nodes, Acanthosis



- CVS: murmurs, gallops
- Lungs/ chest: breast development, gynecomastia, adventitious sounds
- Ab: Tenderness, organomegaly, constipation, scars

- Extremities- hands, cutting (forearms, thighs), scars
- Back- tenderness, SCOLIOSIS, costovertebral tenderness
- Neuro- reflexes, weakness, gait/ ataxia

Assessing for Scoliosis

- If scoliosis is detected use a scoliometer to test for the degree of scoliosis
- **An angle greater than 7° is the threshold for further workup or referring to a specialist**
- **Adolescent growth spurt greatest risk for worsening scoliosis**

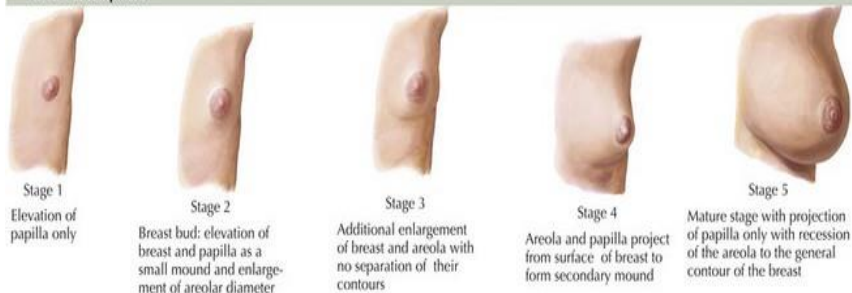


Assessing Pubertal Development

- Breast/ genitalia exam with chaperone (parents aren't chaperones)
- Puberty onset
 - 8 to 13 years in Females (Average age is 11 years)
 - 9 to 14 in males (Average age is 12 years)
 - Males on ave usually start puberty 2 yrs later than females
- Growth spurt for girls happens early in puberty (tanner 2-3), menses tanner 4,6, Boys happens later in puberty.

Puberty: Tanner Staging

Breast development



Female pubic hair development



Male pubic hair and genital development

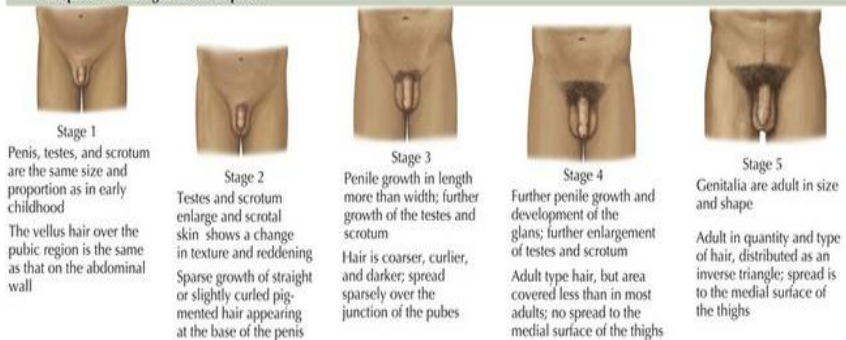


Table 24-1. Tanner Stages of Development

	Female	Both	Male
Stage	Breast	Pubic hair	Genitalia
I	Preadolescent	None	Childhood size
II	Breast bud	Sparse, long, straight	Enlargement of scrotum/testes
III	Areolar diameter enlarges	Darker, curling, increased amount	Penis grows in length; testes continue to enlarge
IV	Secondary mound; separation of contours	Coarse, curly, adult type	Penis grows in length/breadth; scrotum darkens, testes enlarge
V	Mature female	Adult, extends to thighs	Adult shape/size

Tanner Stages

Sexual maturity rating (Tanner stages) of secondary sexual characteristics

Boys – Development of external genitalia
Stage 1: Prepubertal
Stage 2: Enlargement of testes and scrotum; scrotal skin reddens and changes in texture
Stage 3: Enlargement of penis (length at first); further growth of testes
Stage 4: Increased size of penis with growth in breadth and development of glans; testes and scrotum larger, scrotal skin darker
Stage 5: Adult genitalia
Girls – Breast development
Stage 1: Prepubertal
Stage 2: Breast bud stage with elevation of breast and papilla; enlargement of areola
Stage 3: Further enlargement of breast and areola; no separation of their contour
Stage 4: Areola and papilla form a secondary mound above level of breast
Stage 5: Mature stage: Projection of papilla only, related to recession of areola
Boys and girls – Pubic hair
Stage 1: Prepubertal (the pubic area may have vellus hair, similar to that of forearms)
Stage 2: Sparse growth of long, slightly pigmented hair, straight or curled, at base of penis or along labia
Stage 3: Darker, coarser, and more curled hair, spreading sparsely over junction of pubes
Stage 4: Hair adult in type, but covering smaller area than in adult; no spread to medial surface of thighs
Stage 5: Adult in type and quantity, with horizontal upper border

UpToDate®

- Vaccines (HPV, Tdap, MCV4, Men B)
- Lab work?
 - Lipids testing, Anemia?, PCOS?/ menstrual issues?/ obesity?/ STD testing?
- Referral?
- Medications?
- Imaging?
- Anticipatory Guidance
- Words of wisdom (for both patient and Parents)
- follow-ups

- **Primary care is primary provider**
 - **Screening, often start medications if needed**
- Social work
- Guidance counselor
- Psychologists
- Psychiatrists
- Adolescent specialists
- Hospital for acute care

Early (11-14yrs)

- Puberty starting, starting struggle for independence but still childish in many ways, same sex peer groups, starting interest in sexuality, concrete thinking- (things black and white/ right and wrong)

Discussions:

- Don't dismiss their struggle
- Puberty and changes
- Discuss in concrete terms, minimize analogies/ abstractions
- Safety: Most likely to act without thinking...social media, sex, drugs, vaping
- **Plant seeds**

Middle (15-17 yrs)

- Continued physical changes, independence progressing, love/passions, family conflict highest, mixed peer groups

Discussions:

- Still concrete but may have higher level discussion
- Friends/family/relationship changes
- Love interests/ more mature relationships
- Safety: +driving
- Promote independence and corresponding responsibilities (job)
- Water, stake, plant more seeds

Late (18-21)

- more mature self identity, thinking and independence, peers and family dynamics change, mature relationships, can think more about future and others, understand life may be grey, not as simple as black and white/ right and wrong

Discussions:

- More mature conversation
- Less lecturing, More refinement of ideas, guidance, suggestions
- Guide specific goals and plans about future
- Water, admire the plant, plant more

- **Varies by region/ pediatricians/ patients**
 - **Considerations-**
 - adult medical problems
 - Pediatric medical problems esp syndromes, dev delay
 - Continuity of care
- **Most important for adolescents with SPECIAL HEALTHCARE NEEDS**
 - **IMPROVES THEIR HEALTH OUTCOMES**
 - **IMPROVES QUALITY OF LIFE**
 - **PREVENTS LOSS OF A MEDICAL HOME**
 - **IMPROVES CHRONIC DISEASE CONTROL**

Documenting/Recording a Adolescent Exam

- Please refer to Bates guide to physical examination and history taking (11th ed.). Pages 872-875
- The adolescent **SUBJECTIVE** is more detailed than previous exams given the information you gather from TWO HISTORIANS (THE PATIENT AND THE PARENT). The Objective, Assessment and Plan are the same. Note if discussed alone with patient, document chaperone present for breast/genitalia exam

Summary Slide

- Tailor your questions and discussions based on adolescent developmental stages- Early, Mid, Late
- Non-accidental injury, Suicide, homicide are top causes of mortality, then neoplasm and Heart Disease
- Build rapport with both first, then talk alone and confidentiality/disclaimer statement. Understand and state limits of confidentiality
- HEADS assessment
 - Start with less intrusive H,E,A and look for red flags
 - More personal D and S after rapport built
- Screens: PHQ2,9,CRAFFT,CAGE,GAD2,7
- Course of action if Suicidal ideation, 988, counseling, Hosp if imminent
- Pertinent Physical findings in an adolescent
 - Scoliosis 7degrees on scoliometer need further eval, risk during growth spurt)
- First sign of puberty in female is breast buds (age 8-13), first in males is scrotal enlargement (age 9-14)

Plant a seed, Grow a tree



Guide them thru...Thank You



Lecture Feedback Form:

<https://comresearchdata.nyit.edu/redcap/surveys/?s=HRCY448FWYXREL4R>

References

- References:
 - Bickley, L. S., Szilagyi, P. G., & Hoffman, R. M. (2016). Bates guide to physical examination and history taking (12th ed.). Philadelphia: Lippincott Williams & Wilkins.
 - Bickley, L.S., Szilagyi, P. G (2013). Bates guide to physical examination and history taking (11th ed.). Philadelphia: Lippincott Williams & Wilkins.
 - Lissauer, Tom, MB BChir FRCPH; Carroll, Will, BM BCh MD MRCPCH (2018) Illustrated Textbook of Paediatrics (5th ed.)Elsevier Limited.
 - Bright Futures: Guidelines for Health Supervision of Infants, Children and Adolescents (American Academy of Pediatrics)
 - Kliegman, R. St. Geme III (2020). Nelson Textbook of Pediatrics (21st ed). Elsevier sect 137.2, Ch 138.

Arkansas Law- Adolescent Right to Care

- ARK. CODE ANN. § 20-9-602 (2012). CONSENT GENERALLY
 - (section 7) Any unemancipated minor of sufficient intelligence to understand and appreciate the consequences of the proposed surgical or medical treatment or procedures, for himself or herself (can consent to treatment)
- ARK. CODE ANN. §20-16-508 (2012). CONSENT BY MINOR 24
 - (a)(1) When a minor who believes himself or herself to have a sexually transmitted disease consents to the provision of medical care or surgical care or services by a hospital or public clinic or consents to the performance of medical care or surgical care or services by a physician who is licensed to practice medicine in this state, the consent: (A) Is valid and binding as if the minor had achieved his or her majority; and (B) Is not subject to a later disaffirmance by reason of his or her minority. (2) **The consent of a spouse, parent, guardian, or any other person standing in a fiduciary capacity to the minor shall not be necessary in order to authorize hospital care or services or medical or surgical care or services to be provided to the minor by a physician licensed to practice medicine.** (b) Upon the advice and direction of a treating physician or in the case of a medical staff any one (1) of them, a physician or member of a medical staff may inform the spouse, parent, or guardian of any minor as to the treatment given or needed but shall not be obligated to do so. The information may be given to or withheld from the spouse, parent, or guardian without the consent and over the express objection of the minor.

NEW YORK LAW- Minors Right to Medical Care

- **N.Y. PUB. HEALTH LAW § 2504 (2012). ENABLING CERTAIN PERSONS TO CONSENT FOR CERTAIN MEDICAL, DENTAL, HEALTH AND HOSPITAL SERVICES**
 - 1. Any person who is eighteen years of age or older, or is the parent of a child or has married, may give effective consent for medical, dental, health and hospital services for himself or herself, and the consent of no other person shall be necessary.
 - 2. Any person who has been married or who has borne a child may give effective consent for medical, dental, health and hospital services for his or her child. Any person who has been designated pursuant to title fifteen-A of article five of the general obligations law as a person in parental relation to a child may consent to any medical, dental, health and hospital services for such child for which consent is otherwise required which are not: (a) major medical treatment as defined in [subdivision \(a\) of section 80.03 of the mental hygiene law](#); (b) electroconvulsive therapy; or (c) the withdrawal or discontinuance of medical treatment which is sustaining life functions.
 - 3. Any person who is pregnant may give effective consent for medical, dental, health and hospital services relating to prenatal care.
 - 4. Medical, dental, health and hospital services may be rendered to persons of any age without the consent of a parent or legal guardian when, in the physician's judgment an emergency exists and the person is in immediate need of medical attention and an attempt to secure consent would result in delay of treatment which would increase the risk to the person's life or health.
 - 5. Where not otherwise already authorized by law to do so, any person in a parental relation to a child as defined in section twenty-one hundred sixty-four of this chapter and, (i) a grandparent, an adult brother or sister, an adult aunt or uncle, any of whom has assumed care of the child and, (ii) an adult who has care of the child and has written authorization to consent from a person in a parental relation to a child as defined in section twenty-one hundred sixty-four of this chapter, may give effective consent for the immunization of a child. However, a person other than one in a parental relation to the child shall not give consent under this subdivision if he or she has reason to believe that a person in parental relation to the child as defined in section twenty-one hundred sixty-four of this chapter objects to the immunization.
 - 6. Anyone who acts in good faith based on the representation by a person that he is eligible to consent pursuant to the terms of this section shall be deemed to have received effective consent.